You Deliver Care, We Optimize Revenue.

Uncovering revenue. Strengthening hospital performance.

A revenue identification and recovery partnership for Cook County Health's emergency and trauma care.

HOSPITAL PARTNERS

Northwestern Medicine®

HUMBOLDT PARK HEALTH
ADVANCING HEALTH EQUITY



We work inside the revenue cycle you already run.

01 / CONTEXT

The pressure

Hospitals face reimbursement challenges, rising operational costs, complex payer requirements, and growing uncompensated care obligations.

02 / OPPORTUNITY

The gap

In ED, trauma and other encounters, Workers' Compensation, Third-Party Liability, auto and other responsible-party coverage can go unidentified or unpursued by traditional workflows.

03 / SUPPORT

What LaborForce adds

Specialized payer identification, patient engagement, revenue auditing, payer verification, and lien administration support.

04 / OUTCOME

The goal

Identify additional reimbursement, improve revenue realization, and reduce avoidable administrative work — without replacing current systems or teams.

HOW IT WORKS

01 **Technology screening**

Injury-related encounters are flagged from existing registration and billing data.

02 **Specialist review**

Analysts determine the likely responsible payer and what information is missing.

03 **Direct patient contact**

Dedicated staff confirm the facts and close gaps, respecting each patient's choices.

04 **Follow-through**

Billing corrections, lien support, and payer follow-up until the account resolves.

Revenue that traditional workflows may overlook.

Patients often arrive after workplace incidents, motor vehicle accidents, or injuries involving another liable party. Registration and billing workflows may not consistently capture or resolve:

01 Whether the injury occurred while the patient was working

02 Whether a motor vehicle or other third party may be responsible

03 Whether another insurer or payer should be investigated

04 Whether the hospital has the right claim, attorney, or payer information

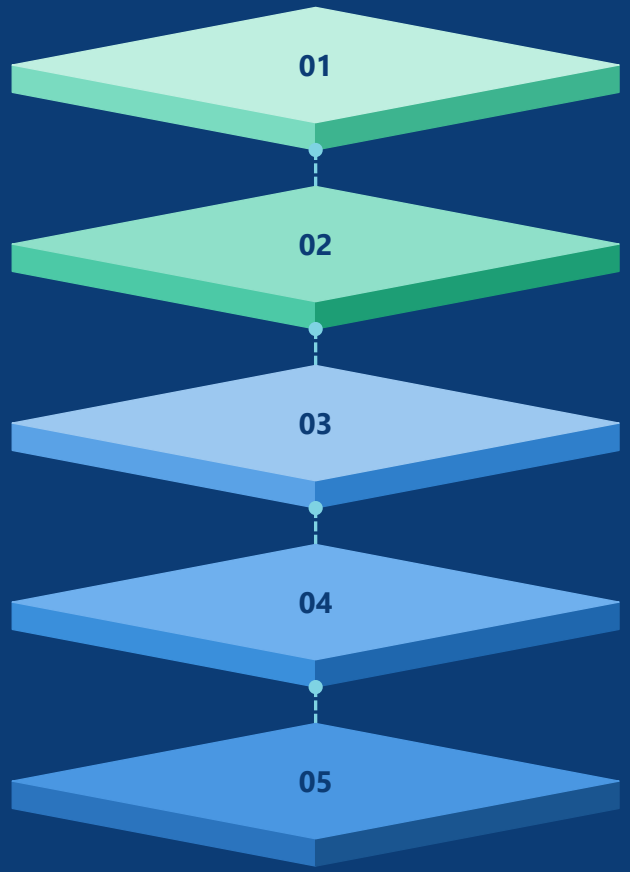
05 Whether a hospital lien is available, documented, and followed up

06 Whether unresolved cases need investigation after discharge



THE CONSEQUENCE Valid reimbursement opportunities may remain unidentified, incorrectly routed, delayed, or insufficiently pursued.

The LaborForce solution.
A specialized operational layer.



01 — Proactive Revenue Identification

Review injury-related encounters, identify potentially responsible payers, and prioritize accounts for investigation.

02 — Dedicated Patient Engagement

Outreach to verify injury circumstances and collect missing claim, employer, insurer, or attorney information.

03 — Payer Verification & RCM Coordination

Coordinate findings with registration, billing, and revenue cycle teams for payer determination and follow-up.

04 — Revenue Auditing & Reconciliation

Monitor unresolved cases, flag missing or inconsistent information, and track case status and reimbursement.

05 — Lien Administration & Recovery

Support approved lien documentation, follow-up, and reimbursement tracking within the legal and contractual framework.

LaborForce works alongside your billing and collections teams.

Turn hidden revenue opportunities into visible results.

PLANNING INPUTS

01 **Annual ED encounters** — **150,000**

25,000 — 200,000

02 **Additional coverage identified** — **6%**

2% — 10%

03 **Average incremental net collection** — **$4,000**

$500 — $5,000

04 **Average ED charge** — **$10,000**

$3,000 — $15,000

ANNUAL INCREMENTAL NET COLLECTIONS

$36M

≈ $3M a month · ≈ $99K a day · $108M over three years

IDENTIFIED ENCOUNTERS / YEAR

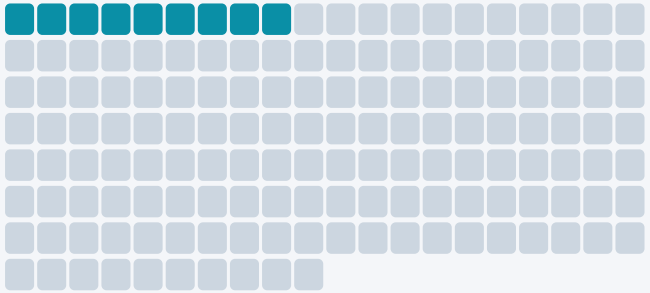
9,000

ASSOCIATED GROSS CHARGES / YEAR

$90M

VOLUME & COVERAGE

A visible share of every visit.

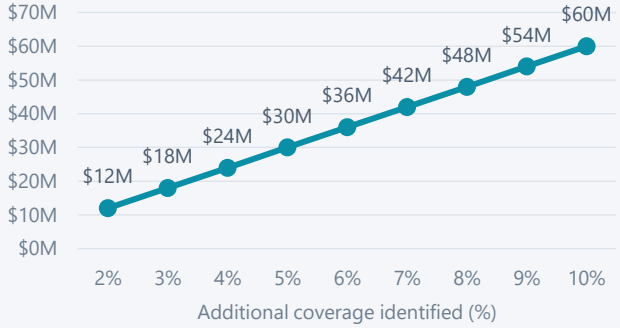


Additional coverage identified — Other encounters

COVERAGE SENSITIVITY

What changes as coverage improves?

$36M at 6% identified coverage



ANNUAL ENCOUNTERS	×	ADDITIONAL COVERAGE	×	NET COLLECTION / ENCOUNTER	=	ANNUAL NET COLLECTIONS
150,000		6%		$4,000		$36,000,000

90-day pilot.
John H. Stroger, Jr. Hospital.

PHASE 01 — **DAYS 1 – 15**

15 of 90 days

Discovery & Baseline

Review current injury-related registration, payer identification, billing, and follow-up. Agree on pilot scope, eligibility criteria, compliance requirements, and baseline measures.

PHASE 02 — **DAYS 16 – 60**

45 of 90 days

Identification & Investigation

Evaluate designated encounters, conduct authorized patient outreach, identify alternative payer sources, and coordinate verified findings with existing hospital teams.

PHASE 03 — **DAYS 61 – 90**

30 of 90 days

Validation & Review

Reconcile outcomes, review billing corrections and collections, assess operational impact, and recommend continued monitoring and potential expansion.

HOW TO READ THE PILOT

Many claims take longer than 90 days to resolve. The pilot measures **leading indicators** and any collections realized; final financial performance is reviewed as the identified account cohort matures.

COOK COUNTY HEALTH PROVIDES

- Executive sponsor and agreed pilot scope
- ED volume, payer mix, and current TPL/WC workflow data
- Compliance requirements and data-access terms

LABORFORCE PROVIDES

- Screening, specialist review, and case investigation
- Dedicated patient outreach and lien administration support
- Joint reporting at days 30, 60, and 90

Transparent. Measurable. Jointly reviewed.

Leading

Early signals that cases are being found and verified

- Encounters screened and flagged
- Number and % with verified additional responsible-party coverage
- Accounts accepted for billing correction or recovery action

Financial

What the program returns, net of adjustments

- Additional collections attributable to the program, net of reversals and adjustments
- Incremental net revenue after program costs

Operational

Effect on hospital teams and account flow

- Staff workload
- Turnaround time
- Unresolved account aging

Quality

Safeguards that stay in place throughout

- Compliance
- Patient experience
- Documentation quality

JOINT REVIEW CADENCE

LAUNCH

Scope, eligibility criteria, and baseline agreed

DAY 30

Baseline confirmed; screening volumes and first flagged cohort

DAY 60

Verification rate, accounts accepted for correction, staff workload impact

DAY 90

Collections to date, cohort maturity, and a joint decision on continuation

Our ask: a 90-day pilot at John H. Stroger, Jr. Hospital.

Strengthen Cook County Health's fiscal resilience while supporting accessible, high-quality care for the community — through a focused, transparent, and measurable pilot, reviewed jointly at days 30, 60, and 90.

01

Confirm sponsor & scope

Executive sponsor and pilot scope at Stroger Hospital

02

Share baseline data

ED volume, payer mix, and the current TPL/WC workflow

03

Agree & launch

Compliance requirements and success metrics; launch Phase 1

LABOR FORCE
MEDICAL MANAGEMENT

Thank you

We look forward to discussing the pilot with your leadership team.